

Vatarakta

Learning Objectives

- ⇒ To learn the concept of Vatarakta with synonyms
- ⇒ To learn the Nidana and Samprapti of Vatarakta mentioned in Ayurveda literatures
- ⇒ To frame out the probable Samprapti Ghataka based on the progression of the disease
- ⇒ To learn Samprapti of Avasthika Vatarakta
- ⇒ To enlist the Vidhi Samprapti of Vatarakta
- ⇒ To learn Purvarupa, Upadrava, Sadhyasadyata of Vatarakta
- ⇒ To know the differences between Sandhigata Vata, Amavata and Vatarakta

Introduction

वातरक्ताभ्यां जनितो व्याधिः वातरक्तं Chakra, Ca.Ci. – 29/2
किंवा वातर एव अवस्थान्तरप्राप्ते वातरक्तम् Chakra, Ca.Ci. – 29/2



Figure 207: Pratyatma Lakshana of Vatarakta

Vatarakta or Vatachonita or Vatasrik is a spectrum of diseases where we may observe the involvement of vitiated Vata Dosha and Rakta Dushti altogether at a particular time. In this disease manifestation, Vata and Rakta are not having any relation unlike Rakta and Pitta. Yet, both Vata Dosha and Rakta Dhatu separately undergo vitiation, but combines together to manifest Vatarakta in Tvak, Sira, Snayu, Asthi and Sandhi as discolouration of skin and pain in joints. As far as the commencement of the disease is

concerned, Vatarakta is considered as Adhyatmika Vikara, Doshabala Pravritta Vikara and Pakvashayasamuttha Vikara and Sharirika Vikara. Primary Srotas afflicted in this disease is Raktavaha Srotas with Sanga and Vimargagamana as the nature of Srotodushti.

खुडं वातवलासाव्यमाख्यातं च नामभिः॥1१॥ Ca.Ci. – 29/11

This disease is having the synonyms as Khuda, Vatabalasa and Adhyavata. In Ayurveda literatures, the term Khuda refers to Sandhi or joints. As joint involvement is observed in this spectrum of diseases, Vatarakta is also known as Khuda Vata.

Vatabalasaka – The Shonita (Rakta) gets strength by impeded function of vitiated Vata. Hence, Vatachonita is also termed as Vatabalasaka.

Adhyavata – as the disease more commonly observed in wealthy people. Hence, the name Adhyavata is used for the disease Vatarakta.

Nidana of Vatarakta

लवणाम्लकारस्निग्धोष्णा जीर्णभोजनैः।

क्लिन्नशुष्काम्बुजानूपमांसपिण्याकमूलकैः॥5॥

कुलत्थमापनिष्पा वशाकादिपल्लेक्षुभिः।

दध्यारनालसौवीरशुक्ततत्रसुरासवैः॥6॥

विरुद्धाध्यशनक्रोधदिवास्वप्नप्रजागरैः।

प्रायशः सुकुमाराणां मिष्टान्नमुखभोजिनाम्॥7॥

अचङ्क्रमणशीलानां कुप्यते वातशोणितम्।

अभिघातादशुद्ध्या च प्रदुष्टे शोणिते नृणाम्॥८॥

कषायकटुतिक्ताल्परूक्षाहारद्वोजनात्।

हेयोष्टयानयानाम्बुक्कीडाप्लवनलङ्घनैः॥९॥

उष्णे चात्यध्ववैषम्याद्वयवायाद्वेगनिग्रहात्।

Ca.Ci. – 29/5-10

The etiological factors mentioned from *Lavana* till *Achankramana Shila* are responsible for the abnormality of *Rakta Dhatu*. From *Kashaya Rasa* substances onwards till *Vega Nigraha* are responsible for the vitiation of *Vata Dosha*.

- ❖ Excess consumption of *Lavana Rasa*¹ (salty taste) substances.
- ❖ Excess consumption of *Amla Rasa*² (sour taste) substances.
- ❖ Excess consumption of *Katu Rasa*³ (pungent taste) substances.

- ❖ Excess consumption of *Kshara*⁴ (alkaline or corrosive) substances.
- ❖ Excess consumption of *Snigdha*⁵ (oily) substances.
- ❖ Excess consumption of *Ushna Virya*⁶ (hot in potency) substances.
- ❖ Excess consumption of *stale substances*⁷.
- ❖ Excess consumption of *dry substances*⁸.
- ❖ Excess consumption of meat of aquatic animals.
- ❖ Excess consumption of meat of *Anupa* animals.
- ❖ Excess consumption of *Pinyaka*.
- ❖ Excess consumption of *Mulaka* (radish)⁹.
- ❖ Excess consumption of horsegram, blackgram, beans, leafy vegetables, meat and sugar cane products.
- ❖ Excess consumption of curds, intoxicating substances, butter milk with *sour taste*¹⁰.

1 लवणः स्तम्भसङ्घातवन्ध विध्यापनोऽग्निं कृत्॥१२॥ स्नेहनं, स्वेदनस्तीक्ष्णो रोचनश्छेद मेदकृत्।

A.Hr.Su. – 10/12,13

सोऽतियुक्तोऽत्रपवनं खलति पलित वलिम्॥१३॥ तृदकुष्टविषवीसर्पान् जनयेत्क्षपयेद्वलन्।

When person consumes excess amount of *Lavana Rasa* substances, that will lead to the manifestation of *Vatarakta*. *Lavana Rasa* is having the Ap and Tejo Mahabhuta in its Panchabhautika composition, may be responsible for the vitiation of *Rakta Dhatu* in due course of time.

2 अम्लोऽग्निदीप्तिकृत्सन्धो हृद्यः पाचनरोचनः। उष्णवीर्यो हिमस्पर्शः प्रीणनः क्लेदनो लघुः॥१०॥

A.Hr.Su. – 10/10-12

सोऽस्यभ्यस्तस्तनोः कुर्याच्छैथिल्यं तिमिरं भ्रमम्॥११॥ पण्डुपाण्डुत्ववीसर्पशोफविस्फोटवृज्वरान्।

Amla Rasa is having *Prithvi* and *Tejo Mahabhuta* in its composition. When an individual consumes more amount of *Amla Rasa* substances will lead to the increase of *Tejo Mahabhuta* in the body, leading to the vitiation of *Rakta Dhatu* and thereby it may be a triggering factor for the manifestation of *Vatarakta*.

3 The Panchabhuta composition of *Katu Rasa* are *Vayu* and *Tejo Mahabhuta*. When an individual consumes more amount of *Katu Rasa* substances leading to the affliction of *Rakta Dhatu*, leading to the manifestation of *Vatarakta* if conducive environment for the progression of the disease is observed.

4 As per *Sushruta*, the qualities of *Kshara* substances are *Ushna*, *Tikshna*, *Tejo Mahabhuta* dominant. In its therapeutic dosage, consumption of *Kshara* substances will be responsible for the pacification of all *Tridosha*, but excess consumption of *Kshara* substances will vitiate *Pitta Dosha* and this may vitiate *Rakta Dhatu* in due course of time in case of conducive environment.

5 *Snigdha* (unctuousness) is one of the qualities of *Pitta Dosha*. if a person consumes unctuous substances in excess will lead to the vitiation of *Pitta Dosha* in the body. Based on the concept of *Ashraya Ashrayi Bhava*, vitiation of *Pitta Dosha* may indirectly indicates the affliction of *Rakta Dhatu* in the body.

6 As per *Ayurveda* literatures, *Ushna* (hot) quality is one of the important quality of *Pitta Dosha*. Therefore, excess consumption of *Ushna Virya* substances such as spicy substances will lead to vitiation of *Pitta Dosha* which in turn leads to the affliction of *Rakta Dhatu* in the body.

7 Excess consumption of *stale substances* implies that these food substances will remain in the stomach for 2 hours following closure of pyloric antrum soon after the food intake. This may lead to either inflammatory response in the stomach or vomiting which occurs due to reverse peristaltic movement or both.

- This will lead to the inadequate conversion of macromolecules into micromolecules, thus macromolecules may lead to the infection in the stomach region as we are consuming more carbohydrate rich diet.
- As *stale foods* are being taken, the infective process may occur even rapidly.
- This will lead to poor digestive fire (weakened gastric juice, inadequate conversion of pepsinogen to pepsin and incomplete conversion of macromolecules to intermediate molecules or micromolecules), thus the person may have gastric mucosal damage.
- As the macromolecules or intermediate molecules present in the stomach, they will pass through damaged gastric mucosa easily into interstitial compartment of stomach. This will further move into the lymphatic system and later enters into the superior vena cava after entry into the intravascular compartment at the junction of left internal jugular vein and left subclavian vein.
- Thus, this may lead to the circulation of incompletely transformed food substances into the blood stream, thus, leading to impurity of blood.

8 When an individual consumes excess quantity of *dry substances*, that will lead to the increase of *Vata Dosha* in the body. Due to this, there will be shrinking of either intravascular or interstitial or intracellular compartments or any two compartments or all compartments may happen. When there is shrinking of intravascular compartment occurs, this may lead to slowing of blood circulation. When there is slow blood circulation, that may lead to initiation of sludging of blood.

9 बालं दोषहरं, वृद्धं त्रिदोषं, मारुतापहमं स्निग्धसिद्धं, विशुष्कं The qualities of radish are as mentioned in *Ca.Su.* – 27/168. Fresh radish will alleviate *Dosha*; overgrown radish will vitiate all three *Dosha*; if prepared with *Snigdha* substances, then this will decrease *Vata Dosha* in the body, dry radish will decrease *Kapha* and *Vata* vitiation.

10 These substances are having *Ushna Virya* and *Amla Vipaka* and thereby responsible for the vitiation of *Pitta Dosha* in the body and thus results in the affliction of *Rakta Dhatu* in due course of time.

- Consumption of food substances which are having mutually opposite qualities i.e. *Viruddhahara*
- Consumption of food substances without the digestion of previously consumed meal¹¹
- Anger¹²
- Sleeping during day time¹³
- Awakening at night time¹⁴
- The people who are susceptible to have physiological disturbances for minor changes in their environment (*Sukumara*)
- The people who are having *sedentary lifestyle*¹⁵

The Vata vitiating factors

- Excess consumption of *Kashaya Rasa*¹⁶ (astringent taste) substances
- Excess consumption of *Tikta Rasa* (bitter taste) substances
- Excess consumption of *Katu Rasa* (spicy) substances

- Excess consumption of *little amount of food*¹⁷
- Excess consumption of *Ruksha (dry) substances*¹⁸
- Frequent fasting*¹⁹
- Frequent travel in *camel, horse*²⁰
- Frequent swimming, playing, jumping
- Too much *physical stress*²¹
- Excess walking in *summer season*²²
- Excess sexual intercourse in summer season
- Excess suppression of *natural urges*²³

Samprapti of Vatarakta

वायुर्विबुद्धो वृद्धेन रक्तेनावारितः पथि॥१०॥

कृत्स्नं संदूषयेद्रक्तं तज्ज्ञेयं वातशोणितम्।

Ca.Ci. – 29/10-11

- First, there will be vitiation of *Vata Dosha*
- Rakta Vridhi* leading to the obstruction in the *Srotas*
- Manifestation of *Vatashonita* due to the simultaneous involvement of vitiated *Vata* and *Rakta*

11 When the person consumes the food substances before the digestion of previously ingested meal is termed as *Adhyashana* as per Ayurveda fundamentals. When an individual is habituated to such food pattern for longer time, that will lead to the transformation of ingested food into *Shuktata* or *Amlibhavatva* or acidification of consumed food. This leads to the vitiation of *Pitta Dosha* and thus may lead to the affliction of *Rakta Dhatu* in due course of time.

12 *Krodha* or anger is said to be responsible for the vitiation of *Pitta Dosha* and if a person frequently showing irritability, the person may have severe degree of *Pitta Dosha* vitiation and thus the person may have affliction of *Rakta Dhatu* in due course of time.

13 In this situation, we may have the physiological changes occur in the body similar to the effects occur in case of *Adhyashana*.

- Macromolecules will not be converted into micromolecules because of poor digestive fire (weak gastric juice, poor conversion of pepsinogen into pepsin and gastric mucosal damage because of infection which occurs as a result of bacterial proliferation on carbohydrate rich diet taken by human beings.
- As gastric damage occurs, macromolecules may move into interstitial compartment of stomach, and later take the course of lymphatic system, as these are macromolecules, which bypass the portal system.
- As these macromolecules are heavier in molecular weight, they move slowly in the intravascular compartment, thus impair the circulatory function.

14 In this situation, deposited fat in the adipose tissue start to enter into the intravascular compartment, thereby chances of atheroma in the blood vessels are more.

15 When the individual is exposed to these aetiological factors, there will be energy input will be more than energy expenditure. That means there will accumulation of nutrients, deposition of fat in the adipose tissues of the breast, buttocks, subcutaneous tissues and cheek starts to occur. In the meantime, there will be circulation of lipid content in the intravascular compartment. This may lead to atheromatous changes in the blood vessels. Thus, the person may have symptoms of peripheral arterial disease in due course of time.

16 As per Ayurveda literatures, excess consumption of *Kashaya*, *Katu* and *Tikta Rasa* substances is said to be responsible for the vitiation of *Vata Dosha* in the body.

17 Frequent consumption of little amount of food leads to the increase of *Laghu* quality in the body. We know that *Laghu* quality is one of the quality of *Vata Dosha*. Thus, frequent consumption of little amount of food leads to the vitiation of *Vata Dosha*.

18 Excess consumption of *Ruksha (dry) substances* leads to the increase of *Ruksha* quality in the body. *Ruksha* quality is the main quality of *Vata Dosha*. Thus, based on the concept of *Samanya* and *Vishesha* concept, there will be increase of *Ruksha* quality in the body leading to the increase of *Vata Dosha* in the body.

19 Frequent fasting leads to the similar effect which can be seen in case of consumption of little amount of food.

20 These factors will lead to the increase of *Chala* quality in the body. If the person is performing these activities in excess, will lead to the increase of *Laghu* quality too. Thus, there will be the vitiation of *Vata Dosha* in the body.

21 Too much physical stress leads to the increase of *Vata Dosha* in the body. When an individual is undergoing too much physical stress, will lead to the depletion of body nutrients. Initially, there will be depletion of carbohydrates, followed by depletion of fats and proteins. Thus, too much physical stress for longer duration may lead to the emaciation of body and thus we can observe *Vata Dosha* vitiation in the body.

22 In exposure to above etiological factors for longer duration, similar effects of exposure to excess physical stress can be observed.

23 When an individual suppresses the natural urges, will lead to the vitiation of *Vata Dosha* in the body. Let us take the example of suppression of *Purisha* (faecal matter), *Adhovata* (flatus) and *Mutra (urine)*, there will be vitiation of *Apana Vata*. This will lead to the reverse movement of *Apana Vata*, leading to the affliction of *Samana Vata* and later there will be affliction of *Prana Vata* in due course of time. When there is affliction of *Samana Vata*, the person may be presented with abnormal functioning of *Agni* leading to *Avipaka* (indigestion).

तस्य स्थानं करौ पादावस्यः सर्वसन्धयः।

कृत्वाऽऽदी हस्तपादे तु मूलं देहे विधावति॥1 2॥

सौक्ष्म्यात् सर्वसरत्वाच्च पवनस्यासुजस्तथा।

तद्रयत्वात् सरत्वाच्च देहं गच्छ सिराः॥1 3॥

पर्वखभिहतं क्षुब्धं वक्रत्वादवतिष्ठते।

स्थितं पित्तादिसंसृष्टं तास्ताः सृजति वेदनाः॥1 4॥

करोति दुःखं तेष्वेव तस्मात् प्रायेण सन्धिषु।

भवन्ति वेदनालाला अत्यर्थे दुःसहा नृणाम्॥1 5॥ Ca.Ci. – 29/12-15

- ⊗ The main parts involved in *Vatashonita* are joints all joints of the body, joints of upper and lower limbs in particular.
- ⊗ First of all, the affliction starts in smaller joints of either upper or lower limbs
- ⊗ Due to *Sukshmatva* and all pervading nature of *Vata*, *Dravatva* and *Saratva* of *Rakta Dhatu*, the abnormal *Vata* and *Rakta* will move all over the body.
- ⊗ Stasis of the abnormal *Vata* and *Rakta* occurs in the joints due to the complexity of the structure of the joints.
- ⊗ Due to the stasis of *Vata* and *Rakta*, there will be various kinds of discomfort ranging from mild to severe forms of discomforts.

Samprapti Ghataka

- ⊗ **Dosha :** *Vata*
- ⊗ **Dushya :** *Rakta*
- ⊗ **Srotas:** *Raktavaha Srotas*

- ⊗ **Srotodushti Prakara :** *Sanga and Vimargagamana*
- ⊗ **Agni :** *Jatharagnimandya, Raktadhatvagnimandya*
- ⊗ **Ama:** *Jatharagnimandyaajanya* and *Raktadhatvagnimandyaajanya*
- ⊗ **Udbhava Sthana :** *Pakvashaya*
- ⊗ **Sanchara Sthana:** *Sarva Sharira*
- ⊗ **Vyakta Sthana :** *Sandhi*
- ⊗ **Rogamarga :** *Abhyantara, Bahya and Madhyama*
- ⊗ **Adhishtana :** *Sharirika*
- ⊗ **Vyadhyavastha :** *Ashukari*
- ⊗ **Sadhyasadhya :** *Krichrasadhya to Asadhya*

Purvarupa of Vatarakta

स्वेदोऽत्यर्थं न वा कायं स्पर्शाशत्वं क्षतेऽतिरुक्।

सन्धिष्वैथिल्यमालस्यं सदनं पिडकोद्गमः॥1 6॥

जानुजङ्घोरु कट्यं सहस्तपादाङ्गसन्धिषु।

निस्तोदः स्फुरणं भेदो गुरुत्वं सुप्तिरेव च॥1 7॥

कण्डूः संधिषु ग्भूत्वा भूत्वा नश्यति चासकृत्।

वैवर्ण्यं मण्डलोत्पत्तिर्वाताकपूर्वलक्षणम्॥1 8॥

Ca.Ci. – 29/16-18

- ⊗ Excess sweating
- ⊗ No sweating²⁴
- ⊗ Blackish discolouration²⁵
- ⊗ Absence of tactile perception²⁶
- ⊗ Severe pain on trauma to the body²⁷
- ⊗ Looseness of the joints

24 It is being said that Vatarakta is having the vitiation of Vata Dosha and vitiation of Rakta Dhatu simultaneously at a particular point of time. When there is extreme affliction of Rakta Dhatu, poor vitiation of Vata Dosha, the person may have excess sweating. The reason for this statement is there is Ashraya and Ashrayi Bhava with Pitta and Sveda.

- When there is more vitiation of Vata Dosha than affliction of Rakta Dhatu, the person may have absence or negligible amount of sweating.
- As per current mechanism of physiology of sweating, this is occurring as a result of peripheral vaso dilatation. This occurs due to the heat produced during the metabolism of nutrients in the body. During the metabolism, heat is generated. This is one of the mechanism of fever manifestation. The other mechanism of fever causation is prevention of heat loss.
- Heat is generated because of following reasons.
 - o Basal rate of metabolism of all cells of the body
 - o Extra rate of metabolism caused by muscle activity, including muscle contractions caused by shivering
 - o Extra metabolism caused by the effect of thyroxine (and to a lesser extent, other hormones such as growth hormone and testosterone) on the cells
 - o Extra metabolism caused by the effect of epinephrine, norepinephrine & sympathetic stimulation on the cells
 - o Extra metabolism caused by increased chemical activity in the cells, especially when the cell temperature increases
 - o Extra metabolism needed for digestion, absorption, and storage of food
- Heat loss (reduced heat loss from the body) leads to the increased temperature in the body. Two factors determine the rate of transfer of heat to the exterior:
 - o How rapidly heat can be conducted from the site where it is produced in the body core to the skin?
 - o How rapidly heat then be transferred from the skin to the surroundings?
- In the current situation, the abnormality may be present in muscle or connective tissue etc, there will be extra metabolism in these tissues and this is leading to the increase in heat generation and this may lead to the increased sweating from the body
- When there is hampered heat loss from the body, this means there will be peripheral vaso constriction, this may lead to absent or negligible sweating from the body.

25 This is considered to be a type of primary skin lesion, i.e. macules.

26 This is attributable to damage to sensory receptors of the skin, especially the patient will have pathology in the integumentary system in the patient of Vatarakta.

27 When an individual is having the prodromal symptoms of Vatarakta, if the patient is exposed to trauma to the lower limb, that will release

- Laziness²⁸
- Sluggish movement²⁹ of the body parts
- Appearance of blisters³⁰ in different parts on the skin
- Pricking type³¹ of pain in knee, waist, thighs, joints of upper and lower limbs
- Pulsation³² in above said body parts
- Tearing type³³ of pain in above said body parts
- Heaviness³⁴ in above said body parts
- Numbness³⁵ in above said body parts
- Itching around the affected joints
- Discolouration³⁶ of the skin, circular elevation in the skin in affected areas

Sankhya samprapti of Vatarakta

उत्तानमथ गम्भीरं द्विविधं तत् प्रचक्षते।

खड्गसांश्रयमुत्तानं गम्भीरं त्वन्तराश्रयम्॥19॥

Ca.Ci. – 29/19

तत्र वातेऽधिके वा स्याद्रके पित्ते कफेऽपि वा।

संसृष्टेषु समस्तेषु यच्च तच्छृणु लक्षणम्॥24॥

Ca.Ci. – 29/24

3 types depending upon involvement of tissues in the body

1. Uttana,
2. Gambhira and
3. Ubhayashrita

8 types depending upon the Dosha involved in the manifestation of the disease,

1. Single Dosha dominant – 3 Vataja, Pittaja, Kaphaja
2. Dual Dosha dominant – 3: Vata-pittaja, Kapha-pittaja, Vata-kaphaja

3. Triple Dosha dominant – 1: Sannipataja
4. One Rakta dominant – 1: Rakta Dhatu involved in the manifestation

Uttana Vatarakta

कण्डूदाहुरुगायामतोदस्फुरणकुञ्चनैः।

अन्विता श्यावरक्ता त्वग्वाद्ये ताम्रा तथेय्यते॥20॥

Ca.Ci. – 29/20

If the *Dosha*³⁷ involved in between *Tvak* and *Mamsa*, then we can say that the disease is in the stage of *Uttana Vatarakta*. The *Lakshana* observed during this stage are

- Itching
- Burning sensation
- Pain
- Stretching of the skin
- Pricking type of pain
- Abnormal pulsation

Gambhira³⁸ Vatarakta

गम्भीरे श्वयथुः स्तम्भः कठिनोऽन्तर्भृशार्तिमान्।

श्यामस्तानोऽथवा दादतोदस्फुरणपाकवान्॥21॥

रुग्विदाहान्वितोऽभीक्ष्णं वायुः सन्ध्यस्थिमज्जसु।

छिन्दन्निव चरत्यन्तर्वक्त्रिकुर्वेध वेगवान्॥22॥

करोति स पट्टे या शरीरे सर्वतम्वरन्।

Ca.Ci. – 29/21-23

If the vitiated *Vata* and *Rakta* are observed in the tissues deeper than *Mamsa*, i.e. *Medas* onwards, then we can infer the *Gambhira* stage of *Vatarakta* in an individual.

- Hard swelling
- Stiffness of the joints
- Severe excruciating pain inside the joints
- Blackish discolouration of the skin

the inflammatory markers in the affected part. But, due to the poor functioning of the blood vessels of the lower limb, healing process of the wound may be delayed. This may lead to severe pain as there is delay in healing process.

28 This is considered as subjective complaint. This can be attributable to sludged blood which can occur in the capillaries, where in transport of proteins, WBCs etc will occur from the capillaries into the interstitial compartment and intracellular compartment.

29 This is attributable to the sludged blood in the capillaries, where in more movement of fluid with proteins will move from intravascular compartment to interstitial and intracellular compartments. As more amount of proteins are present in the interstitial and intracellular compartments, this will lead to the increase in oncotic pressure in the interstitial compartment, which further leads to the movement of fluid into the interstitial compartment. Thus, there will be heaviness in the body parts where pathological events are happening. This will lead to the sluggish movement of the body parts.

30 Blisters are the skin lesions having fluid accumulation within or under the epidermis. This has many causes, exact clinical diagnosis must be based on a closed study of physical signs. Morphology and distribution of the bullous eruption helps in accurate diagnosis. E.g. in case of pemphigus, the patient will have flaccid bullous eruptions within epidermis which will lead to painful erosions after rupture of bullae.

31 Whenever, there is pricking type of pain, it implies that there is a possibility of inflammatory reaction happening inside the body parts where pathological events are happening.

32 Increased pulsation in above said body parts implies that increased vascularity in above said body parts. In other words, there may be a possibility of development of collaterals in the above said body parts.

33 Tearing type of pain may imply the underlying process of infective pathology.

34 Refer the context of sluggish movement of body parts of this chapter.

35 This is attributable to the destruction of sensory receptors when there is pathological process is happening in the integumentary system.

36 These are attributable to the worsening of the pathological events in the lower limb secondary to the development of infection or ischemia of the affected part of the body.

37 In *Uttana Vatarakta*, the vitiated *Dosha* will remain in superficially situated *Dhatu* i.e. *Tvak*, *Rakta* and *Mamsa*.

38 In this type of *Vatarakta*, the vitiated *Dosha* will remain in deep situated *Dhatu*.

- ⊗ Coppery discolouration of the skin
- ⊗ Burning sensation
- ⊗ Pricking type of pain
- ⊗ Abnormal pulsation
- ⊗ Suppuration
- ⊗ Pain and burning sensation in the bones, joints and bone marrow as cutting type
- ⊗ *Khanja* – loss of movement of both lower limbs
- ⊗ *Pangu* – loss of movement of one lower limb

Ubhayashrita Vatarakta

सर्वेधि विज्ञेयं पातासगुभयाश्रयम्॥2 3॥

Ca.Ci. – 29/23

In this type of *Vatarakta*, both *Lakshana* of *Uttana* and *Gambhira* stages of *Vatarakta* are seen.

Vataja Vatarakta

विशेषतः सिरायामशूलेस्फुरणतोदनम्।

शोथस्य का क्षयं च श्यावतावृद्धिहानयः॥2 5॥

धमन्यङ्गुलिसन्धीनां सङ्कोचोऽङ्गोऽतिरुक्।

कुञ्चनस्तम्भने शीतप्रद्वेषचानिलेऽधिके॥2 6॥

Ca.Ci. – 29/25-26

- ⊗ Dilatation of the vessels
- ⊗ Pricking type of pain
- ⊗ Abnormal pulsation
- ⊗ Needle prick type of pain
- ⊗ Blackish discolouration of swelling
- ⊗ Dryness of the skin
- ⊗ *Dhamani Sankocha* (Vaso constriction)
- ⊗ *Anguli Sankocha* (constriction of fingers)
- ⊗ *Sandhi Sankocha* (constriction of joints)
- ⊗ Stiffness of the body parts
- ⊗ Severe pain in the joints while movement of the joints
- ⊗ The patient will have aversion to cold environment

Raktaja Vatarakta

श्रयथुर्भृशरुतोदुस्तानधिमिचिमायते।

स्निग्धरुक्षैः शमं नैति कण्डूक्लेदान्वितोऽसृजि॥2 7॥

Ca.Ci. – 29/27

- ⊗ Swelling is associated with severe pain – pricking type of pain
- ⊗ Coppery discolouration of the skin
- ⊗ Tingling sensation of the body parts
- ⊗ Discomfort manifested will not subside by adopting either *Snehana* or *Rukshana* therapy

Pittaja Vatarakta

विदाहो वेदना मूर्च्छा वेदस्वप्ना मदो भ्रमः।

रागः पाकध मेदस्य शोषश्चोक्तानि पैत्तिके॥2 8॥

Ca.Ci. – 29/28

- ⊗ Burning sensation in the affected parts
- ⊗ Discomfort in joints
- ⊗ Altered state of consciousness
- ⊗ Excess sweating
- ⊗ Thirst
- ⊗ Intoxicating feeling
- ⊗ Giddiness
- ⊗ Redness of the affected parts
- ⊗ Pus formation in the affected parts
- ⊗ Tearing type of pain in the affected parts
- ⊗ Excess dryness of the affected parts

Kaphaja Vatarakta

स्तैमित्यं गौरवं स्नेहः सुप्तिर्मन्दा च रुक् कफे।

Ca.Ci. – 29/29

- ⊗ Abnormal feeling similar to the sensation observed when a person is covering wet cloth over his body
- ⊗ Heaviness of the body parts
- ⊗ Unctuousness of the affected parts
- ⊗ Numbness of the affected parts
- ⊗ Mild pain of the joints

Samsrishta and Sannipataja Vatarakta

हेतुलक्षणसंसर्गाद्विद्याद्वन्द्विदोषजम्॥2 9॥

Ca.Ci. – 29/29

3 types of *Samsrishta* types of *Vatarakta* usually show mixed *Lakshana* of *Dosha* vitiation and *Sannipataja* type of *Vatarakta* usually show *Lakshana* of all three *Dosha* vitiation.

Upadrava of Vatarakta

अस्वप्नारोचकश्वासमांसकोथशिरोग्रहाः।

मूर्च्छाय मदरुक्त्वाज्वरमोहप्रवेपकाः॥3 1॥

हिक्कापाङ्गुल्यवीसर्पपाकतोदभ्रमक्लमाः।

अङ्गुलीवक्रता स्फोटा दाहमर्मग्रहार्बुदाः॥3 2॥

एतैरुपद्रवैर्वर्ज्यै मोहेनैकेन वाऽपि यत्।

संप्रस्त्रावि विवर्णे च स्तब्धमर्बुदकृश्च यत्॥3 3॥

वर्जयेच्चैव संकोचकरमिन्द्रियतापमम्।

Ca.Ci. – 29/31-34

⊗ *Asvapna*³⁹ (Loss of sleep)

⊗ *Arochaka*⁴⁰ (tastelessness)

39 When an individual is presented with chronic inflammation in vessels of lower limb, rest pain is one of the main clinical features of the disease, and it is associated with disturbed sleep or complete loss of sleep.

40 When an individual is suffering from chronic disease related to cardiovascular system pertaining to lower limb, shift of the blood occurs from different parts of the body including alimentary canal towards lower limb. This mechanism may lead to poor secretion in the alimentary canal including salivary secretion. This in turn leads to the poor mixing of salivary secretion to the bolus of the food, thereby the person may have tastelessness.

- *Shwasas*⁴¹ (breathing discomfort)
- *Mamsakotha*⁴² (putrefaction/ necrosis of the tissue)
- *Shirograha*⁴³ (stiffness of the head region)
- *Murcha* (altered state of consciousness)
- *Mada*⁴⁴ (confusion)
- *Ruk*⁴⁵ (pain)
- *Trishna*⁴⁶ (thirst)
- *Jvara*⁴⁷ (raised body temperature)
- *Moha*⁴⁸ (stupor or confusion)

- *Pravepaka* (involuntary movements)
- *Hikka*⁴⁹ (hiccup)
- *Pangulya*⁵⁰ (lameness)
- *Visarpa*⁵¹ (cellulitis)
- *Paka*⁵² (suppuration)
- *Toda*⁵³ (pricking type of pain)
- *Bhrama*⁵⁴ (giddiness)
- *Klama*⁵⁵ (physical exhaustion without physical exercise)

When an individual is suffering from chronic debilitating disease, that may lead to the depletion of body nutrients in the sequential manner of carbohydrate, fat and protein. When this depletion occurs, human body switches over to fat metabolism in the adipose tissue and muscle nutrients in the muscular tissue which lead to formation of ketosis and lactic acidosis respectively. The whole process is termed as metabolic acidosis. Subsequently, the person may develop respiratory alkalosis, one of the clinical feature observed in this occasion is breathing discomfort.

This is attributable to hampered blood supply to the lower limb observed in peripheral arterial disease or thrombo angiitis obliterans. In this situation, the person may have decreased oxygen and nutrient supply leading to necrosis or gangrene formation. It indicates the severity of disease process.

This can be attributable to electrolyte imbalances, hyponatremia in particular. In the context of Mada, Murcha, the causes and clinical features of hyponatremia is described in this chapter.

This can be attributable to the development of electrolyte imbalances, hyponatremia in particular. This is a clinical condition where sodium ions are deficient in the blood which is caused by excess diuretic use, diarrhoea, heart failure and renal disease. Clinically, hyponatremia is identified by headache, confusion and stupor; seizures and coma may occur.

This can be attributable to either intermittent claudication or rest pain or both. This occurs in the individual suffering from peripheral arterial disease due to the accumulation of P substance owing to inadequate blood flow. The site of the pain is depends upon the level of arterial occlusion.

- Rest pain is continuous and aching in nature. This occurs due to ischaemic changes to the somatic nerves. It is considered as cry of the dying nerves. This is usually worse at night, gets aggravated by elevation of the leg above the level of heart and is relieved by hanging the leg below the level of the heart. It usually affects the most distal part first that means the tip of the toe. The painful part becomes very sensitive and any movement or pressure causes an acute exacerbation.

This is attributable to decreased body fluid either in intravascular compartment or in interstitial or in intracellular compartments or in any two or in all three compartments. Refer the context of mechanism of thirst mentioned in Nidana Prakasha written by the author for better understanding of thirst.

This is usually attributable to development of raised body temperature owing to the development of necrosis or gangrene (developing infection in the affected part where blood flow is either stopped or reduced).

- When the person develops cellulitis (bacterial infection of the skin), the person may develop fever.
- Refer the context of Mada and Murcha explained in this chapter.
- The patient who has irritation of diaphragm will have hiccup. This is caused by sudden involuntary contraction of both the diaphragm and the external intercostal muscles, in association with rapid glottic closure. This usually occur with a frequency of 4 and 60 per minute.
- Though various causes mentioned for the manifestation of hiccup i.e. intrathoracic diseases including mediastinal, diaphragmatic and intra abdominal disease.
- Central causes such as epidemic: encephalitis, intracranial diseases, hysteria etc.
- In case of the persons who are suffering from chronic diseases of lower limb may develop uremia. This condition is said to be responsible for manifestation of hiccups.

The person who is suffering from severe peripheral arterial disease, this may lead to the necrosis or gangrenous changes, this may lead to the further management i.e. amputation of the affected limb. Once amputation is done, the person has to have a state of Pangulya (unless he fits the amputated part with artificial limb).

This is attributable to poor blood supply to the lower limb which results in stasis of the blood in the blood vessels. This is responsible for bacterial infection to occur of the subcutaneous tissue of the skin. When this occurs, the person may develop Visarpa. This is attributable to body's response to the virulence of microorganisms. When neutrophils and macrophages engulf large number of bacteria and necrotic tissue, essentially all neutrophils are going to die. After several days, a cavity is often excavated in the inflamed tissues, that contains varying portions of necrotic tissue, dead neutrophils, dead macrophages and tissue fluid. This mixture is commonly known as pus.

- In the patient suffering from Vatarakta, this pus formation mechanisms in lower limbs, in other words, pathogen might have entered in the lower limbs.

The term Toda refers to pricking type of pain. When the individual is having inflammatory reaction, one of the symptom in terms of inflammatory response.

This is attributable to hypovolemic shock. This occurs either due to loss of blood cell mass or from loss of plasma volume. This occurs as a result of poor blood supply to brain (syncopal episode).

This is attributable to poor blood supply to the skeletal muscles. When an individual is suffering from peripheral arterial disease in the lower limb, this is attributable to the hampered blood supply to the lower limb following physical exercise owing to reduced

- *Anguli Vakra*⁵⁶ (deformities of the fingers)
- *Sphota*⁵⁷ (eruptions)
- *Daha*⁵⁸ (burning sensation)
- *Marmagraha*⁵⁹ (affliction of vital parts)
- *Stabdhatva*⁶⁰ (stiffness)
- *Sankocha*⁶¹ (contractures)
- *Indriyatapana*⁶² (disturbed function of *Indriya*)
- *Vivarna*⁶³ (discolouration)
- *Samprasravi*⁶⁴ (oozing from the affected parts)
- *Arbuda*⁶⁵ (hard swelling like malignant tumours)

Sadhyasadyata of Vatarakta

एकदोषानुगं साध्यं नवं, याप्यं द्विदोषजम्।

त्रिदोषजमसाध्यं स्याद्यस्य च स्युरपद्रवाः॥३०॥

Ca.Ci. – 29/30

अकृत्स्नोपद्रवं याप्यं साध्यं स्यान्निरुपद्रवम्॥३४॥

Ca.Ci. – 29/34

Differences between Sandhigata Vata, Amavata and Vatarakta

Sandhigata Vata	Amavata	Vatarakta
Vata Dosha is mainly involved	Ama associated with vitiated Dosha is involved	Vitiated Vata along with affliction of Rakta is involved
Any joint involvement is seen	Any joint involvement can be seen	Initial affliction of joints of Kara and Pada are seen
Snehana (oleation therapy) improve the situation	Snehana worsens the situation	Snehana worsens the situation
Agnimandya need not be an essential factor	Agnimandya is an essential factor	Agnimandya is an essential factor

blood supply secondary to peripheral arterial disease of lower limb.

- 56 This is attributable to deformities developed in the fingers which occurs as a result of poor functions of the joint and fingers. Let us take the example, if an individual is suffering from the pain in the ankle joint, it is obvious that the person will try to avoid the movement of the painful ankle joint.
- The presence of pain in the ankle joint which occurred insidiously implies the undergoing inflammatory pathological process in the ankle joint. This indicates the increased blood supply to the affected ankle joint.
 - As the person is having inflammation of the ankle joint, and reduced movement of the ankle joint, the healing will occur as fibrosis of the affected joint.
 - When fibrosis of ankle joint occurs, this will be responsible for the altered function of the ankle joint and surrounding joints, leading to the presenting complaints of either limping or pain of the surrounding joints etc. As toes are adjacent to metatarsal joints, which are nearer to ankle joint, the impact of dysfunction of ankle joint may afflict even joints of the toes too.
- 57 This is attributable to the manifestation of blisters which occur as a result of infection of subcutaneous tissue of the skin of the lower limb. When compared to the different parts of the body, lower limb is most frequently afflicted body part to the soil or contaminated water. These will lead to the exposure of body part to the microbes present in the soil or contaminated water, leading to the formation of blisters in the immunocompromised patient.
- 58 This can be attributable to infection happening in the lower limb or any part of the body where infection is occurring, this will lead to the increased blood supply and thereby there will be interaction between neutrophils and macrophages with microorganisms leading to the formation of pus. Thus, the person may present with burning sensation of the affected part.
- 59 When an individual is suffering from peripheral arterial disease, it implies that poor blood supply to the affected lower limb.
- This will lead to appearance of gangrenous changes if the pathology becomes severe in nature.
 - When the complete occlusion of the artery of the lower limb occurs, that will lead to the gangrene which may spread towards either ankle or knee joint.
 - When the area of ankle or knee joint is afflicted, that will lead to delayed healing of the gangrenous change, as the movement of the joint may delay the healing process. This may lead to even further compromise in the function of the joint.
- 60 The term *Stabdhatva* refers to stiffness of muscles. When stiffness starts to appear, that will lead to reduced function of the painful part. When this event occurs, further this will lead to the poor blood circulation owing the stasis of the blood in the capillaries of the affected part. Thus, the person may have reduced function of the affected organ and later affected part of the body. Stiffness may be observed if the affected part is either skeletal or smooth muscles.
- 61 This can be attributable to the development of contractures which may occur as a result of healing of wound by fibrosis.
- 62 *Vatarakta* is a disease afflicting the upper and lower limbs. The person may develop the pain in the lower limb which may worsen after walking for a shorter period of time (intermittent claudication), in severe case, the person may develop rest pain. Whereas in the upper limb, the person may have bluish discolouration, particularly during cold environment which is associated with severe pain too.
- 63 This can be attributable to discolouration (reddish in initial stage implying inflammation or blackish in case of development of necrosis or gangrene). The former clinical presentation implies the entry of foreign body to the skin or trauma to the body. The latter clinical presentation may imply sudden blockage of blood to the lower limb such as blockage at dorsalis pedis artery or popliteal artery or femoral artery etc. depending upon the clinical presentation, the person may have discolouration will be observed in the patient.
- 64 When an individual is suffering from peripheral arterial disease, the person may develop venous ulcers or stasis ulcers. During this stage, the person may have secretions to increase from the site of ulcer to heal the ulcer.
- 65 When the pathological process of *Vatarakta* is commencing, then more amount of fluid, formed elements, nutrients, proteins move from capillary to interstitial compartment causing sludged blood (slow movement of blood at the venous end of capillary owing to movement of water, nutrients from the capillary to interstitial space). When the pathological process continues in the same tissue, this will lead to enhanced movement of fluid, formed elements, nutrients, proteins from other adjacent capillaries, thus increasing the area of sludged blood. This will lead to increased viscosity of the surrounding tissue, thus leading to the formation of hard swelling.

- The Vatarakta manifested due to vitiation of single Dosha, of recent origin is considered to be Sadhya (easily curable)
- Vatarakta manifested due to the vitiation of two Dosha is considered to be Yasya (manageable)
- Vatarakta manifested due to the vitiation of all three Dosha, associated with Upadrava (complications) is considered to be Asadhya (incurable).
- If Vatarakta associated with Upadrava is considered to be Yasya (manageable) and without Upadrava is considered to be Sadhya (easily curable)

Key summary of the chapter

Chakra, Ca.Ci. – 29/2

- वातरक्त्या जनिता व्याधिः वातरक्तं
- Vatarakta is the disease which manifests in the individual due to vitiation of Vata Dosha along with affliction of Rakta Dhatu. It is also known as Vatashonita, Vatasrik, Khuda, Vatabalasa and Adhyavata.
- When the individual indulges in the exposure to aetiological factors which leads to the vitiation of Vata and affliction of Rakta, the people are Sukumara (delicate people), then such individuals are more prone to suffer from Vatarakta.
- This disease usually start from either Pada (foot) or Kara (hands), then the disease shows diffuse symptoms.
- As far as the types of Vatarakta are considered, 2 types are mentioned based on the Dhatu afflicted: Uttana (involving Tvak, Rakta and Mamsa) and Gambhira (deep seated Dhatu afflicted); 8 types depending upon Dosha vitiation: 3 single Dosha vitiation manifested; 3 Dual Dosha involvement and 1 Sannipataja (triple Dosha involvement and 1 Raktaja (vitiation of Rakta is observed) in the disease process.
- As far as Sadhyasadhya of the disease is concerned, the Vatarakta manifested due to vitiation of single Dosha, of recent origin is considered to be Sadhya (easily curable); Vatarakta manifested due to the vitiation of two Dosha is considered to be Yasya (manageable); Vatarakta manifested due to the vitiation of all three Dosha, associated with Upadrava (complications) is considered to be Asadhya (incurable); if Vatarakta associated with Upadrava is considered to be Yasya (manageable) and without Upadrava is considered to be Sadhya (easily curable)

Self-Assessment: Questions for Review

Long essay questions

- Define Vatarakta, explain Nidana, Samprapti, Bheda, Sadhyasadhya of Vatarakta
- Explain Nidana, Purvarupa, Rupa, Bheda, Upadrava and Sadhyasadhya of Vatarakta

Short essay questions

- Describe Gambhira Vatarakta
- Explain Vidhi Samprapti of Vatarakta

MCQs

1. Which of the following is not the synonym of Vatarakta?

- a. Vatashonita; b. Khuda;
- c. Adhyavata; d. Varchogada

Answer: (d)

2. The pathogenesis starts in which part of the body in case of Vatarakta?

- a. Pada; b. Prishtha;
- c. Lalata; d. Griva

Answer: (a)

3. Akhuvisha is the term associated with which of the following disease?

- a. Amavata; b. Vatashonita;
- c. Amlapitta; d. Kushtha

Answer: (b)

4. Uttana and Gambhira are the varieties of

- a. Amavata; b. Vatarakta;

- c. Jvara; d. Raktapitta

Answer: (b)

5. Sukumara (delicate) people are prone to suffer from

- a. Gridhrasi; b. Vishvachi;
- c. Pakshaghata; d. Vatarakta

Answer: (d)

Chapter resources

- Charaka Samhita, Chikitsa Sthana 29th chapter: Vatashonita Chikitsa Adhyaya
- Ashtanga Hridaya Nidana Sthana 16th chapter: Vatashonita Nidana Adhyaya
- Ashtanga Sangraha Nidana Sthana 16th chapter: Vatashonita Nidana Adhyaya
- Madhava Nidana, 25th chapter: Amavata Nidana Adhyaya
- Golwalla's medicine for students by Aspi F Golwalla and Sharukh A Golwalla, edited by Milind Y Nadkar, 25th edition, Jaypee Brothers Medical Publishers (P) Ltd, New Delhi, 2017
- The Merck Manual of Diagnosis and Therapy by Robert S Porter and Justin L Kaplan, 19th edition, Merck Sharp & Dohme Corp, New Jersey, 2011
- Chamberlain's symptoms and signs in clinical medicine by Colin Ogilvie and Christopher C Evans, 12th edition, Butterworth-Heinemann Publishers, 1997
- Best & Taylor's physiological basis of medical practice by O P Tandon and Y Tripathi, 13th edition, Wolters Kluwer Health (India) Pvt. Ltd, Gurgaon, 2012
- Textbook of Medical physiology by Arthur C Guyton and John E Hall, 11th edition, Saunders Elsevier publications, Philadelphia, 2006
- <https://www.healthline.com/health/systemic-lupus-erythematosus#gallery-open>
